



Delivering the NHS 10-Year Plan today

Turning vision into action

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The NHS's new 10-Year Health Plan: Fit for the Future outlines an ambitious transformation agenda.

It's about shifting care from hospitals into communities, accelerating digital adoption and reorienting the system toward prevention and equity.

[Access the full publication on the NHS website](#)

But the truth is this...

We can't afford to wait 10 years to act.

Patients need better experiences **today**.

Staff need relief from administrative burden **today**.

Systems need efficiency, transparency and financial resilience **today**.

At SPARK TSL, we believe the 10-Year Plan isn't a distant horizon. It's a blueprint we can start delivering right now.

Our role is to help Trusts, Integrated Care Boards and local systems bridge the gap between aspiration and implementation.

That's why I'm proud to share this guide as a resource to drive awareness and inspiration for health authorities as we tackle the changes ahead. It's not just an overview of where the NHS must go. It's a practical roadmap for how technology (when deployed intelligently) can begin creating impact immediately.

In these pages, you'll find:

1. A clear breakdown of the Plan's core pillars.
2. How SPARK TSL's solutions map directly to those priorities.
3. Realistic scenarios showing what delivery looks like in practice.
4. A phased roadmap for implementation, from pilot to system-wide scale.
5. The metrics that matter and how to measure progress.
6. The challenges we must anticipate and the strategies to overcome them.

Above all, though, you'll find a message of urgency and optimism.

The 10-Year Plan calls for radical shifts. With the right partners, those shifts don't need to wait until 2034. They can start transforming patient care as early as this winter.

See how SPARK Fusion® can support this winter

So, this is an invitation.

Let's not wait for the future to arrive. Let's build it, together.

Warm regards,

Jane Stephenson

Chief Executive Officer, SPARK TSL





Introduction: A moment of transformation

The NHS finds itself at a pivotal juncture. Demand is increasing, resources are constrained and patients expect care that's more convenient, connected and personalised. The 10-Year Plan is a declaration that incremental change is no longer sufficient.

There are several key pillars, but the government frames the vision as three radical shifts:



From hospital
to community



From analogue
to digital



From sickness
to prevention

These shifts imply that the NHS of tomorrow will operate under entirely different rules: Fewer avoidable hospital admissions, digital by default, upstream interventions and a more empowered, data-driven workforce.

But transformation isn't something you wait for. It's something you start. Delay means locking in inefficiencies or being forced to retrofit change at greater cost.

The 10-Year Plan: Key themes & pillars

Here's a synthesised overview of the Plan's core pillars and how they set the stage for technology-led transformation...

From hospital to community

- The Plan envisions a **Neighbourhood Health Service**, where more care is delivered in community settings, patients are seen closer to home and outpatient hospital services are reimagined outside of acute settings.
- Hospital-based care becomes more of a “**centre for complexity**,” not the default for every condition.
- The shift reduces pressure on inpatient beds and supports better continuity across social care, primary care and community services.

From analogue to digital

- At the heart is a **Single Patient Record (SPR)**: A unified, interoperable record accessible across care settings.
- The **NHS App** becomes a “front door” to services: Patient booking, referral, data access, advice and triage.
- Digital automation, AI assistants, ambient voice technologies and analytics underpin productivity and reduce administrative burden.

From sickness to prevention

- The Plan targets improving healthy life expectancy and narrowing health inequalities.
- Prevention isn't peripheral; it's central. The Plan calls for genomics, wearables, early risk stratification, behaviour change and joined-up public health.
- Health interventions will be delivered more frequently in collaboration with local authorities, businesses, social care and community partners.

New operating model & a devolved NHS

- Power and decision-making shift away from centralised control toward **Integrated Care Boards (ICBs)** and **Foundation Trusts (FTs)** with greater autonomy.
- Introduction of **Earned Autonomy** regimes, performance regimes and a failure regime for underperformers.
- Some providers will evolve into **Integrated Health Organisations (IHOs)** holding entire health budgets for defined populations.

Transparency, workforce reform, innovation & finances

- **More data, more transparency:** Provider league tables, patient experience metrics and quality reporting via the NHS App and national platforms.
- **Workforce reform:** Shifting education, upskilling in digital & AI, reducing reliance on international recruitment and aligning roles with future care models.
- **Innovation focus:** Data, AI, genomics, wearables, robotics.
- **Financial models** will evolve from block contracts to value-based funding; providers will be incentivised for outcomes, efficiency and innovation.
- **Productivity targets** (e.g. 2% year-on-year gains) and multi-year financial planning (5-year rolling plans) are core.



How SPARK Fusion® aligns with, enables and accelerates the 10-Year Plan

SPARK Fusion® isn't a bolt-on. It's a platform, service and partner ecosystem that naturally aligns with the 10-Year Plan. Let's explore how its features and philosophy map to the Plan's pillars.

Plan pillar	SPARK Fusion® impact
Hospital → community	Enable outpatient, virtual consults, patient self-management, remote monitoring and digital front door capabilities that shift care out of the hospital.
Analogue → digital	Integration with EPR systems, patient apps, data ingestion, interoperability, AI/automation in workflows.
Sickness → prevention	Patient education modules, wellness tracking, analytics to detect high-risk cohorts, early alerts.
Devolved operating model	Flexibility to customise solutions per ICS/Trust, support for local workflows, modular services.
Transparency & patient empowerment	Dashboards, patient-facing feedback, experience reporting, data transparency tools.
Workforce & innovation	Automation to reduce clinician admin load, training & upskilling support for digital tools, embedding new tech (AI, voice etc).
Financial & productivity	Analytics for cost efficiency (food, resource use), outcome-based metrics, support for new funding models and accountability.

Key features (examples):

- ⚡ **Patient app & portal:** Self-service bookings, triage questionnaires, educational content, remote consult scheduling
- ⚡ **Bedside/inpatient modules:** Integrated patient experience, feedback, room control, entertainment, service requests
- ⚡ **Integration layer:** APIs, HL7/FHIR connectivity to EPRs, data exchange across settings
- ⚡ **Analytics:** Real-time dashboards, predictive modeling, process optimisation
- ⚡ **Automation:** Translate the entire platform and education tailored to procedure

By embedding SPARK Fusion® early, Trusts and ICSs can begin realising Plan goals today without waiting for distant system-wide reform.





Use cases and scenarios: Delivering today, enabling tomorrow

To make the vision concrete, here are some illustrative use cases...

1. Acute Trust facing bed pressure & overstay

Context: A large hospital is struggling with over-occupancy, inefficient discharge and rising length-of-stay.

SPARK TSL deployment:

- Integrate with EHR to surface discharge readiness metrics at the bedside.
- Provide information so patients understand their discharge criteria, expected timelines and next steps.
- Use integrations with key providers to find suitable care facilities.
- Enable virtual follow-up and remote monitoring post-discharge, freeing beds.

Outcome & alignment: Supports “hospital → community” shift, reduces avoidable stay, aligns with value-based funding goals.

2. Community/integrated care setting

Context: An ICS wants to reduce emergency admissions from frailty, chronic disease or social isolation.

SPARK TSL deployment:

- Deploy patient-facing apps with regular wellness check-ins, risk questionnaires.
- Multiple modern communication tools from instant messaging to video calling.
- Trigger alerts to community teams based on feedback provided from the patient.
- Link to a single patient record so community clinicians see hospital data.

Outcome & alignment: Advances prevention efforts, strengthens community care, reduces pressure on acute services.

3. Patient empowerment & prevention

Context: A Trust aims to improve outcomes for high-risk patients (e.g. diabetic, cardio, COPD).

SPARK TSL deployment:

- Education modules and behaviour nudges delivered via app.
- Integration with interpreter services to ensure no message gets lost.
- Feedback loops and surveys to engage patients in their care plans.
- Analytics to track patient interactions with content.

Outcome & alignment: Embeds the “sickness to prevention” ethos, supports narrowing health inequalities and positions the organisation as a proactive system.

Implementation roadmap & best practices

Rolling out in service of the 10-Year Plan isn't as simple as flipping a switch. It's about deliberate, phased progress.

Phases:

1. Discovery & strategy (0–3 months)

- Stakeholder alignment (executive, clinical, IT, community).
- Gap analysis vs. Plan objectives.
- Use-case prioritisation.

2. Pilot & proof-of-concept (3–9 months)

- Deploy in constrained domain (e.g. one ward, one outpatient specialty).
- Monitor metrics, adapt workflows.
- Build early champion stories.

3. Scale & integration (9–24 months)

- Expand to full hospital, multiple care settings.
- Integration with digital backbone, EHR, ICS systems.
- Strengthen data flows, governance.

4. Embed & evolve (24+ months)

- Continuous improvement.
- Expand use to prevention, advanced analytics, AI.
- Governance, sustainment.

Best practices & success factors

- ✓ Strong governance & executive sponsorship are critical.
- ✓ Co-design with frontline staff to ensure solutions solve real pain points.
- ✓ Interoperability-first mindset to avoid data siloes.
- ✓ Change management, training and support built in.
- ✓ Iterative feedback loops to refine use.
- ✓ Flexibility — design for modular growth as the NHS Plan evolves.



Measuring impact & ROI

To demonstrate value, measure across multiple domains:

Clinical/quality metrics

- ⚡ Readmission rates.
- ⚡ Length-of-stay.
- ⚡ Avoidable admissions.
- ⚡ Compliance with discharge criteria.

Operational/process metrics

- ⚡ Time to discharge.
- ⚡ Staff time saved (documentation, admin).
- ⚡ Workflow throughput.
- ⚡ System response times.

Financial metrics

- ⚡ Cost per patient, cost avoidance.
- ⚡ Reduction in waste (e.g. food, resource usage).
- ⚡ Return on investment/payback timeframe.

Patient experience

- ⚡ Net Promoter Score (NPS), satisfaction surveys.
- ⚡ Engagement levels (app usage, module completion).
- ⚡ Patient-reported outcomes/ experience measures (PROMs/ PREMs).

Use dashboards, periodic reviews and benchmarking (pre vs. post). As the NHS Plan emphasises transparency and outcomes, these will feed into ICB-level and national reporting.

Risks, challenges & mitigations

A guide must be realistic. Here are likely challenges and how to mitigate them:

Risk/challenge	Mitigation/strategy
Legacy systems, data siloes	Use standards (FHIR, APIs), phased integration, middleware layers.
Resistance to change/culture	Engage early, co-design, champions, continuous communication.
Data privacy, security, consent	Robust governance, role-based access, compliance with UK GDPR, audit trails.
Evolving regulatory or policy environment	Maintain flexibility, modular architecture, stay plugged into policy changes.
Budget constraints/competing priorities	Start small, show early ROI, de-risk through pilots.
Interoperability across ICS boundaries	Build alliances, share standards and data contracts, foster trust.

The NHS 10-Year Plan is a bold, ambitious blueprint, but it's only as good as its implementation. The gap between aspiration and delivery must be closed by organisations that act today.

As a strategic partner, SPARK TSL's technology roadmap, implementation ethos, and domain experience align directly with the Plan's pillars:

- Begin shifting care from hospitals to communities.
- Accelerate digital maturity and interoperability.
- Embed prevention, patient empowerment and transparency.
- Free up clinical time and improve productivity.
- Lay the foundation for outcome-based models and value-based care.

Next steps

- 1. Engage your executive, clinical and digital leaders in a "10-Year Plan readiness" workshop.**
- 2. Conduct a gap/maturity assessment.**
- 3. Identify a high-value pilot use case.**
- 4. Form a cross-functional team and secure funding.**
- 5. Start small, learn and scale.**



A final message from Jane

“The NHS 10-Year Plan is bold in scope, but its success depends on the actions we all take now.

At SPARK TSL, we’re committed to being more than a technology provider. We’re a partner in helping you deliver meaningful change for patients, staff and communities.

If what you’ve read here resonates, I’d love to continue the conversation. Let’s explore how we can turn your challenges into opportunities and start delivering the future NHS today.

Please do get in touch with us to arrange a discussion or demonstration — the first step toward lasting transformation begins now.”

Speak to us

